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Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
7500 Security Boulevard
Baltimore, MD 21244-1850

Thank you for the opportunity to respond to the Request for Information regarding accelerating the adoption and use of Artificial Intelligence (AI) as part of clinical care. Please accept the following comments from Cityblock Health, Inc. and Cityblock Medical Practice, P.A. (Cityblock). Cityblock offers a fully integrated solution that directly delivers clinical care to some of the highest-risk Medicaid and dually eligible populations with complex needs. Powered by advanced technology that provides its care team with a data-driven understanding of member needs and risks, Cityblock has demonstrated industry-leading engagement and member retention and has achieved meaningful reductions in avoidable hospital readmissions and reduced total cost of care. Cityblock currently serves more than 100,000 members, and partners with national and regional Medicaid and Medicare Advantage health plans and health systems across more than ten states.

[What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?](#)

Federal AI Preemption

Issue: State-specific AI regulation creates a patchwork system that increases administrative burden, introduces new complexity, disincentivizes innovation, and limits scalability of AI tools.

Proposal: Release preemptive federal guidance, similar to HIPAA, and / or work with Congress to create legislation that creates a national framework for AI usage in healthcare. This would align well with steps the Administration has already begun to take via the AI Action Plan.

- For example, the December 2025 Executive Order (EO), *Ensuring a National Policy Framework for Artificial Intelligence*, focuses on centralizing AI policy at the federal level to prevent a patchwork framework of state-specific AI regulations. This EO created an AI Litigation Task Force within the Department of Justice to identify and challenge state regulations that inhibit AI innovation and adoption.
- Establishing a healthcare specific AI framework in conjunction with the Task Force would encourage provider adoption by providing a clear roadmap for compliance and best practices in clinical settings.

Collection of Consent for AI/HIPAA

Issue: There is currently no guidance clarifying how patient consent can be obtained for providers to use AI tools for Treatment, Payment, and Operations (TPO) purposes that are subject to HIPAA requirements. The lack of clarity regarding consent collection for AI tools could inhibit provider organizations from implementing AI tools or default to the most restrictive use due to perceived legal or compliance risk.

Proposal: Release federal guidance on collection of patient consent for providers to use AI tools in their care, including any applicable data sharing and communication as well as clinical care delivery, for TPO purposes. For example, federal guidance could allow providers to collect, as a component of HIPAA consent, patient consent that includes use of AI tools.

HIPAA AI Models - Zero Data Retention vs PHI Redaction

Issue: Currently, no federal guidance exists regarding how data must be stored by third-party vendors (i.e. contractors that are not HIPAA “covered entities” and that are storing or processing data for providers), e.g. if Zero Data Retention policies are required versus PHI redaction. This lack of clarity limits innovation by adding complexity and duration when setting up partnerships between AI companies and providers, and can disincentivize collaboration between those entities. The ambiguity also increases risk of patient harm if data are stored improperly - including if PHI is passed into AI models, retained in perpetuity, and potentially used as training data which could lead to unauthorized access to or use of PHI.

Proposal: Release federal guidance that provides direction for providers working with third-party technology companies regarding data retention requirements.

Accurately categorize PHI and non-PHI data sets even when data is integrated for VBC use

Issue: Once a healthcare entity ingests any identifiable data, that data is then treated as PHI once linked to PHI even if it is not related to health. This classification creates a barrier to integrated care, incentivizing organizations to silo data to prevent less sensitive information from being governed by the most restrictive sharing requirements. For example, under current data sharing requirements, providers are prohibited from sharing a patient’s location with a transportation service provider even if the provider is organizing transportation for the patient. This is especially burdensome for value-based care (VBC) providers that are invested in offering evidence-based prevention and must gather information related to access to nutritional food, transportation, etc to accurately assess and address patients’ needs.

Proposal: Create a carve-out for value-based care providers to allow data that does not meet federal PHI criteria at 45 CFR § 164.514 to remain classified as Personally Identifiable Information (or non-PHI), even after being ingested by a VBC provider. This will allow VBC providers to better collaborate with non-medical community partners by making it easier to share data without it automatically being treated like PHI, and reduce the administrative burden for both the providers and the community-based organizations. This would also create greater flexibility for provider-patient communication, since non-PHI is not subject to the same robust sharing requirements as PHI. For example, if non-PHI data could be more easily shared, organizations could tailor patient communication to account for their individual concerns, such as sending personalized directions to a clinic via public transportation for members who do not have access to reliable transportation.

TCPA HIPAA Exemption Extended to use of AI Generative Voice Tech

Issue: The ATDS (automatic telephone dialing system) definition used in the Telephone Consumer Protection Act (TCPA) was developed before the advent of AI voice technology, leaving ambiguity of how AI voice technology should be regulated. Without clear guidance, providers default to the strictest interpretation of the TCPA HIPAA exemption, which risks limiting innovation with AI voice technology. This uncertainty can hinder adoption of sophisticated AI voice technologies, such as automated outbound scheduling and administrative reminders, which could otherwise reduce provider burden.

Proposal: Preserve the TCPA HIPAA exemption and expand it to include use of AI generative voice technology for non-clinical purposes, like appointment reminders and other simple healthcare messaging. AI generative voice technology has the potential to reduce administrative burden and allow providers to focus solely on care delivery, versus administrative tasks.

CMS Clarity/ Sandbox - Frontier/ New Model Research

Issue: There is no federal regulatory clarity or guidance for healthcare providers who want to work with early stage frontier models. This puts healthcare providers at a disadvantage compared to other industries, as AI companies are incentivized to focus on industries with clearer guidelines for testing and use. States are beginning to propose legal frameworks for frontier models with potential healthcare impact; creating a patchwork system by state further complicates innovation and scalability for AI models.

Proposal: Release federal guidance allowing experimental authority for healthcare providers to develop sandbox environments as a testing ground for frontier models. Leverage best practices from other examples working with sensitive data such as employing zero data retention policies.

[What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the](#)

effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Patient Engagement and Communication

Issue: Existing security regulations limit usability of generative AI for personalized communication, e.g. text messages to assist with patient engagement. Specifically, 45 CFR Part 160 and Part 164 (The HIPAA Security Rule) limit the ability to reference PHI and leverage generative AI for meaningful conversations, reminders, and engagement through text messages and other methods absent a secure and encrypted platform.

- Existing regulation dates back to the 2000s and was not designed for the current technology landscape and AI capabilities.
- The original Security Rule was not written at a time when text messaging was a preferred channel of conversations, therefore limiting a modern patient's ability to engage with a medical provider absent a secure portal or app. For example, under a strict interpretation of the Security Rule, a patient appointment reminder text message could not include the name of the patient's medical professional.
- Requiring patients to log into specific applications or portals to review care team messages and reminders creates unnecessary barriers and can negatively impact patient engagement. Lower average digital literacy and access to broadband or Wi-Fi could disproportionately affect vulnerable populations like rural communities or low-income individuals.

Proposal:

- Issue federal guidance around common industry practices for communication of clinical information that is not PHI or is not clearly defined as PHI; e.g., example of texting prescription fill information to patients using the first three letters of the drug name.
- Issue federal guidance around the process for patient consent to those communications, including via text and social media, and the opportunity for patients to consent to communication via multiple types of media using a single form or approval.
- As remote monitoring devices continue to grow in popularity and usage, federal guidance around real-time provider-patient communication based on device data will be useful. Modalities such as texting can amplify the advantages of sharing real-time data with providers and lead to faster patient engagement.

42 CFR Part II Data Sharing

Issue: 42 CFR Part II [Final Rule](#), released in 2024, expanded the liabilities for potential harm due to sharing any medical records that may contain substance use disorder. This disincentivizes and creates barriers to data sharing among providers, which can fragment service delivery, leading to inefficient care and poorer health

outcomes.

Proposal: Create an exemption for value-based care (VBC) providers to simplify the data sharing process, improve care coordination, and encourage innovation within VBC arrangements. Integrating behavioral and physical healthcare is fundamental to delivering value-based care. Aligning Part 2 data with existing HIPAA standards will enable a more holistic approach to integrated care and ensure that sensitive health data is protected by the same robust privacy guardrails used across the rest of the healthcare system.

Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Closing Quality Gaps

Issue: Currently, data from Qualified Health Information Networks (QHIN) can be used by providers to supplement existing data for a more comprehensive view of patient care and health care needs. However, supplemental data from QHINs often cannot be used to meet quality metrics and close gaps. The National Committee for Quality Assurance (NCQA) outlines that data used to close quality gaps must come directly from a patient's medical chart, meaning data from a QHIN is not acceptable for this purpose, unless verified through onerous or expensive processes.

Proposal: Release federal guidance updating data verification and attestation processes to minimize administrative friction for providers utilizing QHINs. While QHIN participants currently attest to data validity at the point of origin, downstream users are often required to re-verify that same data when using it to close quality gaps. Centralizing verification authority under a body like CMS or NCQA would create a "verify once, use many" framework, incentivizing robust data sharing and operational efficiency.

Longitudinal Care Delivery and Coordination

Issue: In the short term, integrated care delivery is hindered because key data are unstructured, lack of availability of a truly holistic longitudinal record facilitated by interoperability, and no means to identify the same member across the multitude of records.

Proposal: CMS could encourage interoperability and strengthen the digital health ecosystem by embedding robust data-sharing expectations into future guidance, ensuring that information follows the patient across the continuum of care.

- With enhanced interoperability, VBP providers like Cityblock can shorten time to market, enable

real-world research with large, comparable cohorts, and more easily coordinate care across multiple health systems.

- Innovations such as AI-powered care coordination, AI copilots, and AI first care teams working across fragmented systems represents a transformative opportunity. Aggregating FHIR-based data across medical, behavioral health, long-term services and supports, and community-based organizations could help enable the use of AI to improve continuity and coherence in the member experience, generate actionable care plans, and ensure synchronization across different actors.
- Expanded access to claims data, Admission Discharge Transfer (ADT) events, behavioral health information, medications, lab results, diagnoses, assessments, referrals and social care needs via FHIR based APIs and via national exchange infrastructure will make it easier to identify gaps in care and manage risk more effectively in real time.
- When paired with transparent benchmarking frameworks like HEDIS metrics, interoperability expands from a compliance requirement into a strategic driver for continuous quality improvement and evidence-based care delivery.
- Improved interoperability will also support the ability to turn unstructured care delivery information into discrete signals for ingestion into machine learning and AI models. Consistent, discrete data with standardized definitions helps turn unstructured data into discrete signals making it more ingestible for machine learning and AI models
- Additionally, the inverse is potentially even more impactful, as AI will actually be able to accelerate interoperability by leveraging large language models (LLMs) to execute on feature extraction from unstructured data and smooth over any schema differences between systems. In other words, LLMs are very effective at translating unstructured data into structured data, particularly when there are standardized definitions for structured data points.

What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patient and Caregiver Feedback

Issue: Successful use of AI in healthcare delivery hinges on the active buy-in and trust of both patients and caregivers.

Proposal: Within CMS models, include stakeholder engagement efforts with patients and caregivers as new AI tools are introduced into the healthcare experience. Patients and caregivers have already raised several opportunities for AI to address existing challenges within health care, as well as some concerns regarding the

loss of human connection.

- Patients and caregivers express interest in AI adoption in clinical care to support administrative tasks such as appointment scheduling, identifying in-network providers, and interpreting coverage.
- There's also an opportunity for AI solutions to support with real time translation for non-English language speakers, and plain language explanation of medical information and treatment options.
- Patients are comfortable with technology as a tool to improve continuity and collaboration, for example AI tools that enable care teams to holistically work together and close the loop from prior conversations.
- However, patients and caregivers have expressed concerns about the loss of human connection and empathy with the advent of AI solutions, the ineptitude of AI tools to solve their problems relative to providers, and the biases inherent in algorithms that AI uses.

Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

Population Specific Data for AI Training

Issue: Most AI tools are trained on commercial insurance data or academic medical center populations. The functionality of an AI model is a direct reflection of its training data. AI tools should be trained on population-specific data; models derived from commercial or academic populations that fail to account for the unique socio-clinical profile of Medicaid beneficiaries and dually eligible individuals could exacerbate existing inequities and inefficiencies.

Proposal: CMS could incentivize or fund creation of realistic synthetic datasets that preserve the statistical properties (e.g. coverage gaps, state variation) of real Medicaid data, and publish them openly.

- CMS could also encourage public performance reporting for Medicaid AI tools, including metrics stratified by race, language, disability, and dual-eligible status, to create market pressure for vendors to actually train on Medicaid data.

Provider-Led AI Research

Issue: Investment in AI solutions developed outside of the health care system often need to be retrofit and incorporated into care models by providers, and may not improve pain points fully or at all. AI should be developed by providers within the system, for seamless and effective integration into care delivery.

Proposal: CMS could incentivize and encourage provider-led AI research and development through

demonstrations or the creation of AI Sandboxes that allow providers to test AI tools. These sandboxes could provide a temporary 'safe harbor' to allow for real-world testing of how AI can streamline workflows and improve patient-provider engagement. Providers are best positioned to develop AI tools that make care more rather than less personalized. Innovative providers are already finding ways to use AI to reduce the amount of paperwork doctors have to do, giving patients more time with human providers.

AI Research

Issue: Lack of AI research and data focused on Medicaid settings risks developing tools that do not function as well for, or miss opportunities, to improve Medicaid care models.

Solution: CMS could offer encouragement, or potentially a financial incentive, to evaluate AI within Medicaid settings. Specifically evaluating AI performance in Medicaid settings, where demographics, comorbidity patterns and care utilization differ from other populations, would be helpful.

- Evaluations should measure not just clinical outcomes but also clinician adoption, health equity impacts, patient trust and cost-effectiveness.
- Implementation science research to evaluate workflow integration would be useful.
 - For example, tracking metrics like clinician adoption of tools, alert fatigue, override rates, and patient experience measures would help stakeholders better understand the utility of these solutions in the Medicaid and dually eligible populations.
- An approved framework or guidance on AI safety will likely accelerate provider adoption. This can be developed through research that focuses on creating standardized approaches for AI "labeling" and usage regulations.

Please do not hesitate to reach out if we can be of any assistance. It would be our pleasure to discuss these comments, and we appreciate the chance to share them.

Sincerely,

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